

RxKids Hawai'i - Cost & Benefits by Income Quintile, TY 2028

Statutory eligibility: Medicaid (clause 1) OR income <= 300% FPL incl. expected unborn child (clause 2)

Point estimate - true uncertainty is +/-30-40%. No admin caseload exists to calibrate against; cost hinges on soft pregnancy/infant incidence assumptions. Read with the assumption band.

Fiscal cost (annual)

Total program cost (benefit)	\$31,413,986
Prenatal arm	\$9,897,557
Postnatal arm	\$21,516,428
Administrative load (8%)	\$2,513,119
Appropriation total (benefit + admin)	\$33,927,104
Sampling 90% CI	\$23,875,212 - \$38,952,759
Assumption band	\$18,848,391 - \$40,053,429

Scenario comparison

Take-up held constant (0.90 newborn / 0.83 prenatal); only the eligibility gate and postnatal window change. Cost = annual benefit dollars; appropriation adds the admin load.

Statutory (Medicaid OR <=300% FPL) · 6-mo postnatal	\$4,500	\$31,413,986	\$33,927,104	13,771
Universal · 6-mo postnatal	\$4,500	\$52,145,328	\$56,316,954	22,858
Universal · 12-mo postnatal	\$7,500	\$87,861,306	\$94,890,211	22,858

Potential option - extend postnatal 6-12 months

The program may opt to extend postnatal payments to the full 12 months (Flint's upper bound). Priced here as an optional add-on:

Additional cost (+6 months)	\$21,516,428
Total cost (12-month design)	\$52,930,414

First fiscal year (launch) - 12mo operating, 12mo enrollment ramp

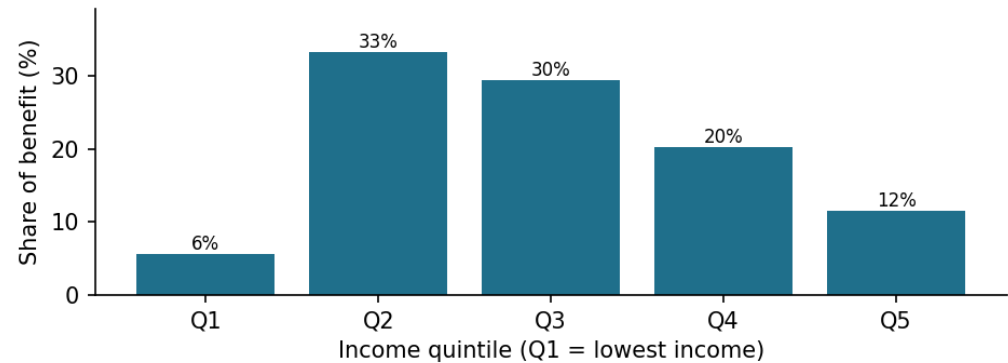
Year-1 cash disbursement, below steady state: enrollment ramps up and the 6-month postnatal caseload takes time to fill.

Year-1 disbursement (total)	\$13,031,385
Prenatal / Postnatal	\$5,361,177 / \$7,670,208
Postnatal deferred to next FY	\$3,984,524
Year-1 as % of steady state	41%
Ramp sensitivity (6 / 12 / 18 mo)	\$20,386,816 / \$13,031,385 / \$8,687,590

Household reach

Eligible families (weighted)	7,340
Expected recipients / year	13,771
Expected pregnancies / infants	6,598 / 7,172
Avg benefit per recipient	\$2,281

Benefit received by income quintile



Quintile	Avg income	Families	Recipients	Total benefit	Share
Q1	\$17,182	110,267	761	\$1,735,257	5.5%
Q2	\$57,028	110,263	4,591	\$10,472,677	33.3%
Q3	\$96,722	110,317	4,059	\$9,260,225	29.5%
Q4	\$154,809	110,231	2,779	\$6,339,862	20.2%
Q5	\$334,087	110,343	1,581	\$3,605,966	11.5%

Cost by county

County	Cost total	Prenatal	Postnatal	Recipients
Hawaii	\$6,793,295	\$2,140,353	\$4,652,942	2,978
Honolulu	\$20,749,567	\$6,537,535	\$14,212,032	9,096
Kauai	\$2,242,739	\$706,616	\$1,536,123	983
Maui	\$1,628,384	\$513,053	\$1,115,332	714

Both arms are driven by OBSERVED births (age-0 dependents in each PUMS tax unit), calibrated to CDC NVSR resident births and projected to the target year with the repo's damped-trend ensemble. Each eligible birth draws one prenatal + one postnatal payment. Cost = the take-up-adjusted benefit weighted by the household (WGTP) weight. Eligibility is tested on MAGI at the tax-unit (MAGI-household) grain, the family Medicaid uses. Benefits-by-quintile rank SPM units on summed income into population-equal fifths. Sampling CI is ACS sampling error (SDR, 80 replicate weights); the assumption band sweeps take-up and the birth count (+/-25%). 2026-2028 FPL is CPI-projected off the 2025 HHS table. Full methodology: RXKIDS_METHODODOLOGY.md.